

Department of Pediatric Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 6.3 Yr/M
Admission No.: anonymous	Department Unit: Pediatric Surgery (Ped Sx Unit 1)	Consultant/Faculty: anonymous
Ward: Pedia Sx 3 C 4	Room/Bed: 3 C 4 Ward/ 3 C 4 Pae 26	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 20-Mar-2023 15:41	D.O.D. : 02-Apr-2023	D.O. Print Report: 08-Feb-2026 15:37

FINAL DIAGNOSIS:

o/c/o penoscrotal hypospadias with uc fistula

OPERATION PERFORMED:

UC Fistula repair and meatal advancement On 02-Apr-2023

SURGEONS: Dr N Borkar

ASSISTANT SURGEONS/SENIOR RESIDENTS: Dr Ashwin

OPERATIVE FINDINGS: Under GA, under AAP, in supine position

No 8 IFT placed. Glans stitch taken

Elliptical incision over uc fistula site and plane created circumferentially and fistula closed over No 8 IFT using 6-0 PDS

Incision at glanular meatus and meatus advanced to glans tip.

Skin flaps elevated and rearranged over neo urethra not overlapping suture line.

Skin closed with 6-0 PDS

Sterile dressing done

CHIEF COMPLAINTS:

o/c/o penoscrotal hypospadias with stage 2 urethroplasty

c/o UC fistula

HISTORY OF PRESENT ILLNESS: As per patient attenders, patient had urethral opening at the scrotum since birth and dribbling of urine from the opening for which he underwent stage 1 hypospadias repair in 4/3/21 and stage 2 repair on 14/7/22. Presently the patient presented with UC fistula at penoscrotal junction.

EXAMINATION FINDINGS :

GENERAL:

PR: 92/M TEMP: 98.2 F

RR: 32/M WEIGHT : 14.4 kg

No Pallor, No Cyanosis, No Clubbing, No Generalised LNE, No Generalised Edema

CNS: WNL

CVS: S1S2 HEARD

RS: BL AIR ENTRY PRESENT. No crepts



PA: Soft non tender, UC fistula present at penoscrotal junction. B/L testes palpable

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
20-Mar-2023 16:51	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.04	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.26	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	8.58	1.0 - 3.0 x10 ³ /μL
		Absolute Monocyte Count	1.39	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	4.22	2.0 - 7.0 x10 ³ /μL
		Basophil	0.3	00 - 01 %
		Eosinophils	1.8	01 - 06 %
		HCT	33.6	34 - 40 %
		Hemoglobin	9.6	11 - 14 g/dl
		Lymphocytes	59.2	44 - 74 %
		MCH	18.5	24 - 30 pg
		MCHC	28.6	31 - 37 g/dL
		MCV	64.9	83.0 - 101.0 fL
		Monocytes	9.6	02 - 10 %
		Neutrophils	29.1	40 - 80 %
		Platelet Count	403	150 - 410 x10 ³ /μL
		RBC	5.18	4.5 - 5.5 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	18.5	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	14.49	4.0 - 10.0 x10 ³ /μL
	Rft	Chloride	104	102 - 109 mmol/L
		Creatinine	0.48	0.9 - 1.3 mg/dl
		Potassium	4.61	3.5 - 5 mmol/L
		Sodium	139	135 - 145 mmol/L
		Urea	29	15.0 - 39.0 mg/dl

***Refer Lab Reports for details.**

HOSPITAL COURSE: Patient is an operated case of penoscrotal hypospadias currently presented with UC fistula. He underwent UC fistula repair and meatal advancement surgery on 23/3/23. Post op period was uneventful, hence being discharged with following advice.

CONDITION AT DISCHARGE: PATIENT IS HEMODYNAMICALLY STABLE. PASSED URINE STOOLS. TOLERATING ORALS

ADVICE ON DISCHARGE:

CIPLOX DROPS L/A BD

SYN IBUGESIC PLUS 5 ML PO BD

SYN LACTULOSE 15 ML PO HS

SYN CYCLENORM 0.01MG PO HS

DISCHARGE BY : anonymous



C-DAC: Centre for Development of Advanced Computing
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91-0120-3063311-13.



DISCHARGE REMARKS :

FOLLOW UP ON : 10-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE :

Summary Prepared By

anonymous

Summary Approved By

anonymous